

Provider Report

Details for

Hospital Discharge Summary

Encounter ID: K509976683

Patient

KAREN CLAYBORNE

Dictation Date

5/28/2026 1:15 PM

Facility

METHODIST HOSPITAL STONE OAK

1139 E Sonterra Blvd

San Antonio TX 78258

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Summary

METHODIST HOSPITAL STONE OAK (COCMSO)
Hospitalist Discharge Summary
REPORT#: 0528-0432 REPORT STATUS: Signed
DATE:05/28/26 TIME: 1315

PATIENT: CLAYBORNE, KAREN UNIT #: K00395352
ACCOUNT#: K509976683 ROOM/BED: K.327-1
DOB: 01/11/66 AGE: 60 SEX: F ATTEND: Dumovich, Jenna DO
ADM DT: 05/26/26 AUTHOR: Dumovich, Jenna DO
RPT SRV DT: 05/28/26 RPT SRV TM: 1315
* ALL edits or amendments must be made on the electronic/computer document *

General Information

Discharge date: 05/28/26
Discharge diagnosis:
Lung adenocarcinoma with metastasis
Hospital course:
See below

Free Text Dx A P Notes

Free text Dx A P notes:

Metastatic lung cancer with multiple parenchymal brain metastases

- Presented with slurred speech and difficulty speaking that started two days prior to admission- MRI shows multiple parenchymal brain metastases with the largest seen in the inferior vermis and cerebellum with associated vasogenic edema within the bilateral cerebellar hemispheres- Patient needs to be restaged at this time- MRI of the CT and L spine with and without contrast - no mets seen
- CT of the chest, abdomen, pelvis with and without contrast - no mets seen- Neurosurgery recommended holding Eliquis
- Increase Dexamethasone to 4 mg q8 hours for vasogenic edema

Type 2 diabetes mellitus, poorly controlled

Hyperglycemia exacerbated by steroids

- A1C is 9.8
- Lantus 15 units BID
- Glucometer ordered

Hypertension

- Patient had urgency in the emergency department, however this has naturally come down
- Resume home meds

Chronic bilateral pulmonary embolisms

- Holding Eliquis at this time given high risk

Right pleural effusion, chronic

-Fu outpatient

Patient medically appropriate for discharge home. Spoke at length with patient and sister in conjunction with Oncology. Plan for discharge home with close outpatient fu with Oncology and Radiation oncology. Teams here have reached out to her oncologist to expedite appointment. Sister is upset and feels that patient is unstable for discharge as she is requiring insulin. Explained that it is not a reason to stay in the hospital. Insulin teaching has been provided. ER return precautions given.

Med Rec

Med Rec

Discharge meds:

Stop taking the following medications:

dexAMETHasone (DECADRON) 4 MG TAB

4 MILLIGRAM ORAL DAILY

hydrALAZINE (APRESOLINE) 10 MG TAB

10 MILLIGRAM ORAL DAILY

Glipizide (Glipizide) 5 MG TAB

0.5 TABLET ORAL TWICE A DAY

APIXABAN (ELIQUIS) 5 MG TAB

1 TABLET ORAL TWICE A DAY

APIXABAN (ELIQUIS) 2.5 MG TAB

2 TABLET ORAL TWICE A DAY

dexAMETHasone (DECADRON) 2 MG TAB

1 TABLET ORAL DAILY

Continue taking these medications:

Osimertinib Mesylate (TAGRISSO) 80 MG TAB

80 MILLIGRAM ORAL DAILY

Nifedipine (Nifedipine ER) 30 MG TAB.SA

30 MILLIGRAM ORAL TWICE A DAY

Carvedilol (Carvedilol) 6.25 MG TAB

1 TABLET ORAL TWICE A DAY

Start taking the following new medications:

ATORVASTATIN (LIPITOR) 40 MG TAB
40 MILLIGRAM ORAL AT BEDTIME
Days = 30 Qty = 30
No Refills

dexAMETHasone (DECADRON) 4 MG TAB
4 MILLIGRAM ORAL EVERY 8 HOURS
Days = 30 Qty = 90
No Refills

SYRINGE NEEDLE,INSULIN, 0.3 ML (INSULIN SYRINGE 31G X 5/16) 100 EACH/BOX
DISP.SYRIN
1 EACH MISCELLANEOUS TWICE A DAY
Qty = 100
No Refills

Blood-Glucose Meter (Accu-Chek Guide Monitor System) 1 EACH EACH
1 EACH MISCELLANEOUS AS DIRECTED
Qty = 1
No Refills

Blood Sugar Diagnostic (Blood Glucose Test Strip) EACH EACH
1 EACH MISCELLANEOUS AS DIRECTED
Qty = 100
No Refills

INSULIN GLARGINE-YFGN (SEMGLEE (YFGN) PEN) 3 ML INSULN.PEN
15 UNIT SUBCUTANEOUS TWICE A DAY
Qty = 30
Refills = 2

Objective

VS/I 0

Last Documented:

	Result	Date Time
B/P	140/88	05/28 1234
B/P Mean	105.6	05/28 1234
Pulse	134	05/28 1234
Pulse Ox	98	05/28 1150
Temp	98.4	05/28 1150
02 Delivery	Room air	05/28 0735
Resp	14	05/28 0735

Free Text Obj Notes

Free Text Obj Notes:

General: Well developed, well nourished, in no apparent distress.

HEENT: Normocephalic, atraumatic. Sclera anicteric. EOMI. Grossly normal hearing. Normal mucosa.

Lungs: Clear bilaterally with normal respiratory effort.

Heart: Regular rate and rhythm, normal S1, S2, no murmurs.

Abdomen: Soft, non-tender, non-distended, +BS.

Musculoskeletal: No deformity.

Extremities: No clubbing, no cyanosis, no edema.

Neurological: No focal deficits, cranial nerves II-XII grossly intact, normal sensation, normal motor.

Skin: Intact without significant lesions or rashes.

Psychiatric: Alert and oriented

Discharge Instructions

PCP

Discharge to: Home/Self Care

Additional Discharge Routines: PCP Follow-Up, Consultant Follow-Up

Diet: Resume Home Diet/Feeds

Follow-up Appointments

PCP follow-up:

PCP:

Gogu, Sudhir R DO

Phone: (210)481-6060

PCP follow up timeframe: In 1-2 weeks

Consulting provider 1:

Provider 1 (free text): Oncology

Consulting provider 2:

Provider 2 (free-text): Radiation Oncology

MIPS/Quality Measures

MIPS Measures MIPS Measures

Quality Measures

Quality Measures Quality Measures

Electronically Signed by Dumovich, Jenna DO on 05/28/26 at 1325

RPT #: 0528-0432

END OF REPORT

